World Health Organization
African Region

Burden- and equity-informed operational planning

Paper MEC-09, policy P7 of 7, for decision

Friday 20 November 2026, 11:20–12:35 | Block C. Use evidence for decisions | For decision

Draft of 4 November 2026 for TWG review. Parameters such as deadlines, response times and thresholds not set by the Framework are proposals, shown as such in the decision options.

The policy in brief

Each Country Office submits a two-page alignment profile with its 2028–2029 workplans, showing how its P/S, funding and workforce match national priorities, the burden of disease and the populations left behind; the Regional Office reviews it before approving the workplan.

Why this policy

The Framework asks leaders to direct resources "toward the Products and Services and populations where the evidence shows they will have the greatest effect", and requires analysis that is sex-disaggregated and equity-sensitive. Planning is the moment where that question is answered, or not.

The alignment method tested in the DRC in September 2026 sets what an office funds, staffs and delivers against national priorities and the burden of disease. It makes the choices visible, so that the office decides cause by cause what WHO takes on, what it leaves to partners and what must be financed.

What the evidence shows

- In the DRC, **91%** of 2026–2027 funding goes to epidemics, polio and immunization, which account for **9%** of the burden of disease; **49%** of DALYs fall in cause groups with no SAM1 P/S (DRC alignment analysis, September 2026).
- Across the 45 countries with GBD 2023 estimates, noncommunicable diseases account for **20% to 77%** of DALYs and injuries for **7% to 17%**: one regional template cannot fit every office.

Scope

The 47 Country Offices; the Regional Office clusters and the DPM in the review of workplans; the MSC Unit for the method and data.

The policy

1. **An alignment profile with the workplan.** With its 2028–2029 workplans, each office submits a two-page alignment profile covering national priorities (the CCS and the national health plan), the burden of disease (IHME GBD, with national data where available), WHO funding and workforce by area, delivery of its priority P/S, and the populations left behind.
2. **A position on each major cause.** For each major cause group, the profile records WHO's position (WHO leads, WHO supports partners, partners lead, or a gap to be financed) and the reason.
3. **Equity.** The profile includes sex-disaggregated and equity analysis and names the vulnerable populations considered (Annex C, Domain 12).
4. **Review before approval.** The Regional Office clusters and the DPM review the profile with the workplan and record their comments before approval. Differences between burden and funding are explained, not penalized: emergencies and the IHR mandate legitimately shift resources.
5. **A data pack for every office.** By 31 March 2027, MSC provides each office with the profile template and a data pack: burden of disease, SAM1 portfolio, funding and workforce. For Mauritius and Seychelles, absent from the GBD 2023 export used, the pack draws on national cause-of-death statistics and WHO Global Health Estimates.

6. **Use, not ranking.** Profiles inform dialogue with Member States and partners, resource mobilization and the next CCS. They are not used to compare or rank offices.

Who does what

WHO Representative	Owns the positioning decisions and presents them to the government and partners.
MEAL focal point and planning officer	Prepare the profile from the data pack.
Programme leads	Contribute the analysis for their areas.
MSC Unit	Issues the method, template and data packs; synthesizes the profiles for the MEC.
Regional Office clusters and DPM	Review profiles with workplans before approval.

Timeline

When	Milestone
31 Mar 2027	Template and data packs sent to all 47 offices
2027 planning cycle	Profiles prepared with the 2028–2029 workplans
Before approval	Regional Office review of each profile with its workplan
MEC 2027	Regional synthesis of the profiles

How compliance is measured

Indicator	Baseline	Target
Country Offices submitting a profile with their 2028–2029 workplans	1 tested (DRC)	47
Profiles reviewed before workplan approval	Not yet measured	47
Profiles recording a position on each major cause group	Not yet measured	47

Decision options

Option	What the MEC would adopt	Implications
A Recommended	**Profile mandatory for all Country Offices with the 2028–2029 workplans**	Every workplan is set against the burden; adds a review step to planning.
B	Mandatory for offices preparing a new CCS in 2027–2028; voluntary for others	Lighter, but most 2028–2029 workplans are approved without it.
C	Guidance and data packs only	No added requirement; alignment stays at each office's discretion.

Resources

MSC analysis to prepare 47 data packs; review time in the clusters during planning.

> **The MEC is asked to**
>
> Adopt the alignment profile as a requirement for all 2028–2029 workplans (option A).

Framework references: Framework Parts I.4, II.2 and X.3; Annex A, Sub-dimensions 2.1, 2.4, 7.3 and 12.1; Annex C, Domains 2 and 12; Annex F, Section F1. Domains: D2 Results architecture; D7 Analytics; D12 Gender and equity.